TO: Centers for Medicare & Medicaid Services, Department of Health and Human Services

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FROM: Helsesoft LLC / GreenLight PA, Bellevue, Washington

RE: Comment on the 2026 CMS Interoperability Standards and Prior Authorization for Drugs Proposed Rule (CMS-0062-P)

I. Introduction and Commenter Identity

Helsesoft LLC is a Washington State health technology company and the developer of GreenLight PA, a prior authorization intelligence platform built specifically for dermatology. GreenLight PA maintains a structured, machine-readable library of prior authorization (PA) criteria for dermatology biologics — including agents such as dupilumab, tralokinumab, risankizumab, and secukinumab — mapped at the drug, insurer, and state level across major commercial payers including Medicare Advantage organizations, Medicaid managed care plans, and commercial QHP issuers. The platform is EHR-independent and contains no protected health information (PHI) by architectural design, enabling deployment across practice settings of all sizes.

Helsesoft LLC was founded by a physician with direct experience in healthcare workflow design and clinical operations, and has built GreenLight PA in close collaboration with practicing dermatologists. The company's development work has required sustained, direct engagement with payer coverage policy documents across multiple drug-indication combinations, insurers, and states — giving us ground-level technical understanding of how prior authorization criteria are published, structured, updated, and applied in practice. This comment reflects that operational experience.

We strongly support CMS's proposal to extend prior authorization interoperability requirements to drugs. The sections below address specific provisions of the proposed rule and respond to two of CMS's explicit Requests for Information.

II. Support for Extending Electronic PA Requirements to Drugs

Relevant sections: II.B (Electronic Prior Authorization for Drugs); II.C (Improving Communications and Decision Timeframes); 91 FR 19890

The 2024 Interoperability and Prior Authorization final rule (CMS-0057-F) made meaningful progress on non-drug PA but created an anomalous gap for specialties like dermatology where virtually all high-cost, high-efficacy treatments are biologics dispensed under pharmacy or medical benefits. For a dermatology practice, the practical effect of CMS-0057-F was limited: the most burdensome authorization workflows — for drugs like dupilumab, which routinely

requires multi-step documentation of prior treatment failure across multiple insurers — remained entirely outside its scope.

CMS-0062-P closes that gap. We specifically support the following proposed provisions:

- **Section II.B.3:** Incorporating drugs covered under a medical benefit into the Prior Authorization API for all impacted payers. Biologics for dermatologic conditions are frequently billed under the medical benefit (Part B for Medicare Advantage), making this provision essential for dermatology.
- **Section II.B.5:** Requiring support for the NCPDP Formulary and Benefit Standard, which will enable providers and their health IT tools to query formulary information and real-time coverage status for covered drugs.
- **Section II.C.2:** Requiring a specific reason for denial in response to PA requests for all drugs. In our experience maintaining payer policy documentation for dermatology biologics, denial reasons as currently communicated are frequently generic and insufficient to guide resubmission.
- **Section II.C.3:** Proposed shorter decision timeframes aligned across MA, Medicaid, CHIP, and QHP programs. Standardizing timeframes across program types significantly reduces administrative complexity for practices managing patients across payer types.
- **Section II.C.7:** Requiring public reporting of PA metrics for drugs. This is the transparency foundation on which downstream accountability mechanisms can be built.

III. A Critical Gap: Published Criteria vs. Real-World Approval Behavior

Relevant sections: II.C.7 (Public Reporting of Drug PA Metrics); III.D (Request for Information: Step Therapy); 91 FR 19890

We write to draw CMS's attention to a structural problem in the prior authorization ecosystem that the proposed rule's transparency provisions begin to address but do not fully reach: the divergence between payers' published prior authorization criteria and the criteria actually applied in approval and denial decisions.

In maintaining GreenLight PA's structured policy library — which monitors coverage policy documents for dermatology biologics across more than 200 drug-insurer-state combinations — we have observed systematic patterns in which published payer criteria do not reliably predict actual approval behavior. These patterns include:

- Step therapy sequences documented in published coverage policies that are inconsistently enforced in practice, with some insurers approving requests that bypass stated requirements while denying others that appear to satisfy them.
- Documentation standards described in policy language that differ materially from what is actually required for approval as reflected in denial rationale communications.
- Coverage criteria that are updated in practice — through apparent internal guidance — without corresponding updates to published policy documents, creating a lag during which providers and their tools are operating on outdated information.

These patterns create measurable harm. Providers build PA submissions around published criteria that may not reflect what the insurer actually requires, leading to avoidable denials, resubmission cycles, and delays in patient access to therapy. The public reporting requirements in Section II.C.7 represent important progress, but aggregate approval and denial rate metrics do not directly surface whether payers are applying their own stated criteria consistently.

IV. Specific Recommendations

A. Require Drug-Level and Specialty-Level Granularity in Public PA Metrics

Relevant section: II.C.7 (Proposed Requirement To Publicly Report Prior Authorization Metrics for Drugs); 91 FR 19890

We recommend that CMS require impacted payers to report PA approval and denial metrics disaggregated by therapeutic area or clinical specialty (e.g., dermatology, rheumatology, oncology) in addition to the proposed drug-level reporting. Dermatology biologics carry authorization burden profiles substantially different from other drug categories: they are typically chronic therapies, require documentation of prior treatment failure, and involve step therapy sequences that vary significantly by insurer and state. Aggregate drug-level metrics will obscure this variation.

Specialty-level disaggregation would enable CMS, researchers, provider organizations, and the public to identify areas of disproportionate burden and target oversight accordingly. It would also allow specialty societies to monitor payer performance within their domain — a form of distributed accountability that requires no additional CMS enforcement infrastructure.

B. Require Payers to Publish Machine-Readable, Structured PA Criteria for Covered Drugs

Relevant sections: II.A.4(a)(6) (Da Vinci CRD, DTR, and PAS Implementation Guides); II.B.3 (Prior Authorization API for Drugs Under Medical Benefit); 91 FR 19890

CMS-0062-P requires payers to implement electronic PA APIs enabling providers to submit requests and receive decisions electronically. We strongly support this. However, the ability to submit a PA request electronically does not, by itself, ensure that providers understand what the payer requires for approval. The underlying PA criteria — step therapy requirements, diagnostic thresholds, prior treatment history requirements, quantity limits, and documentation standards — remain largely inaccessible in structured, machine-readable form.

The Da Vinci Coverage Requirements Discovery (CRD) and Documentation Templates and Rules (DTR) implementation guides identified in Section II.A.4(a)(6) represent the right technical direction: they are designed to surface coverage requirements to providers at the point of care. We recommend that CMS strengthen the proposed rule by requiring impacted payers to implement CRD and DTR as mandatory — not merely recommended — components of the Prior Authorization API for drugs. This would:

- Enable providers and health IT tools to surface accurate, payer-specific requirements at the point of care, reducing avoidable denials driven by documentation gaps.
- Create an auditable, structured record of the criteria payers have committed to applying, against which actual decisions can be evaluated.
- Reduce duplicative manual research burden on practices that currently must interpret narrative PDF policy documents across multiple payers to determine what is required for a given drug.

We acknowledge that payers may seek to protect commercially sensitive aspects of their coverage criteria. We do not propose disclosure of proprietary adjudication logic or internal risk models. We propose only that the criteria a treating provider must satisfy to obtain approval be made available in structured, queryable form via the APIs CMS is already requiring payers to build.

C. Response to RFI III.D: Step Therapy

Relevant section: III.D (Request for Information: Step Therapy); 91 FR 19890

CMS's request for information on step therapy (Section III.D) directly implicates the divergence problem described in Section III of this comment. Step therapy is the single most consequential structural feature of drug PA criteria for dermatology biologics, and it is also the area where the gap between published criteria and applied criteria is most pronounced and most harmful.

In our operational experience monitoring payer policy documentation for dermatology biologics, step therapy requirements are the most frequently updated, most inconsistently enforced, and most difficult to interpret elements of published coverage criteria. We offer the following observations in response to CMS's request:

- **Step therapy requirements vary materially across payers and states for the same drug.** A provider prescribing dupilumab for moderate-to-severe atopic dermatitis may face step therapy requirements ranging from one prior topical agent to multiple systemic agents depending on the insurer and the patient's state. Published criteria for the same drug can require meaningfully different prior treatment histories across payers, creating significant administrative burden for practices managing diverse patient populations.
- **Published step therapy criteria are frequently inconsistent with actual approval behavior.** We have observed patterns where payers appear to approve requests that do not satisfy stated step therapy sequences, and deny requests that do — suggesting that internal adjudication guidance diverges from published coverage policy. This divergence is opaque to providers and impossible to anticipate from published documents alone.
- **The proposed metrics reporting requirements do not capture step therapy compliance.** Aggregate approval and denial rates do not reveal whether denials are based on the step therapy requirements documented in published policy, unstated internal criteria, or administrative deficiencies in submission. CMS should consider requiring payers to report denial reasons at a level of granularity that includes the specific criterion cited — including whether step therapy requirements were the basis for denial.

We recommend that CMS use the step therapy RFI to explore requiring payers to: (1) publish step therapy requirements for covered drugs in structured, machine-readable form; (2) include

step therapy criterion citation in denial reason communications; and (3) attest annually that published step therapy requirements reflect criteria actually applied during the reporting period. These measures would address the root cause of much of the PA burden that this proposed rule aims to reduce.

D. Response to RFI III.C: Improving Implementation of Payer API Technology

Relevant section: III.C (Request for Information: Improving Implementation of Payer API Technology); 91 FR 19890

CMS's request for information on improving payer API implementation (Section III.C) raises questions directly relevant to our experience as a health IT developer serving specialty practices. We offer the following observations:

- **EHR-independent middleware tools are essential for broad provider adoption.** The electronic PA API framework proposed in CMS-0062-P is designed to enable exchange between payers and providers or their EHR systems. In practice, many specialty practices — particularly independent dermatology practices, which represent a substantial share of the prescribers of dermatology biologics — operate with EHR systems that have limited or delayed PA API integration. EHR-independent health IT tools that can consume payer APIs and surface PA requirements at the point of care are a critical link in the adoption chain. CMS's implementation guidance should explicitly acknowledge and support this model.
- **API standardization does not eliminate the need for policy intelligence tools.** Even with standardized FHIR APIs in place, providers will need tools to interpret coverage requirements, compare drug options across payers, and guide PA submission content. The intelligence layer — translating structured payer data into actionable clinical guidance — remains a product and workflow challenge that the API framework alone does not resolve. CMS should consider this when evaluating whether the proposed technical requirements are sufficient to achieve the rule's burden-reduction goals.
- **API endpoint publication (Section II.E) will meaningfully improve the developer ecosystem.** The proposed requirement for payers to report and CMS to publish API endpoint information (Section II.E) is a practical improvement that will reduce the discovery burden for health IT developers building tools that consume payer APIs. We support this provision and recommend that CMS require endpoint publication to include API version information and conformance testing results to enable developers to assess compatibility without direct payer engagement.

V. Conclusion

Helsesoft LLC and GreenLight PA strongly support CMS-0062-P's extension of electronic prior authorization and transparency requirements to drugs. The proposed rule addresses a genuine gap in the regulatory framework established by CMS-0057-F and has the potential to meaningfully reduce PA burden for patients awaiting access to dermatology biologics and other specialty medications.

We urge CMS to strengthen the rule by: making the CRD and DTR implementation guides mandatory for drug PA; requiring specialty-level granularity in public metrics reporting; and using the step therapy and API implementation RFIs to develop requirements that address the divergence between published PA criteria and real-world approval behavior. These additions would substantially enhance the rule's impact and create lasting accountability in the drug PA ecosystem.

Helsesoft LLC welcomes the opportunity to provide additional technical input to CMS on any of the issues raised in this comment. We are available for follow-up at the contact information below.

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